nities is also increased. According to a 2020 WHO report, India needs at least 1.8 million doctors, nurses and midwives to achieve—by 2030—the minimum threshold of 44.5 health workers per 10,000 population it prescribes. A pre-print study conducted this year by WHO Europe and the Public Health Foundation of India, which analysed the active health workforce in India, concluded that the country is still 'well below the WHO threshold'. The situation is worse in rural areas where almost two-thirds of the population resides. The Rural Health Statistics (RHS) 2021, released by the Union ministry of health and family welfare, revealed a 7 per cent shortfall of doctors at Primary Health Centres (PHCs) and a 57 per cent shortfall at Community Health Centres (CHCs) across rural and urban areas. The report also indicates a nationwide shortage of over 80 per cent of the required surgeons and paediatricians at CHCs. In urban areas, the shortfall of doctors at PHCs is 9.8 per cent, while at CHCs, it's 34 per cent.

RAISING PUBLIC SPENDING

Indians also face traumatic levels of stress when it comes to healthcare costs. High out-of-pocket expenditure (OOPE) on health pushes about 55 million people into poverty annually, with over 17 per cent of households incurring catastrophic levels of expenditures each year, a March 2022 WHO report estimated. The overall OOPE share had improved over the years to 48.21 per cent in 2018-19 from 69.4 per cent in 2004-05, according to the National Health Accounts (NHA). But it still remains significantly higher than the global average of 18.1 per cent as of 2019, as per the World Bank. For private healthcare settings, the cost of hospitalisation is seven times higher than in government hospitals, revealed the NSS Household Social Consumption statistics for 2017-18. In the early 1950s, the share of the private sector was 8 per cent of the total healthcare market; this has now risen to 70 per cent. "Either the government needs to improve public healthcare systems or bring down costs or grow insurance cover in the private sector," says Dr Raman.

Increasing public spending to 2.5-3 per cent of the gross domestic product (GDP) can also significantly bring down OOPE to 30 per cent of overall healthcare spend from 48.21 per cent, according to the Economic Survey 2023. Currently, India's health budget is at 2.1 per cent of the GDP. It was among the 10 worst-performing countries in terms of prioritising health in government budgets, at both state and central levels, according to the Economic Survey 2021. Healthcare affordability can also be improved through pooled negotiation and price control of critical drugs. Take the example of cancer. A study published last year in the journal *Frontiers in Public Health* found that the annual direct OOPE incurred on cancer treatment per patient was estimated at Rs 3.31 lakh. Diagnostics (36.4 per cent) and medicines (45 per cent) are major contributors to OOPE for cancer. To bring down the cost of cancer medicines, 23 hospitals affiliated to the National Cancer Grid, a network of over 250 cancer centres that treat over two-thirds of India's cancer patients, procure through a pilot-pooled programme. The initiative aims to improve negotiability for 40 high-value oncology and supportive-care drugs. A study on the project, published in a WHO bulletin in June 2023, states that the project resulted in savings of Rs 1,320 crore compared to the maximum retail prices. The savings, it adds, ranged from 23 to 99 per cent (median 82 per cent) and were higher with generics than branded and newly patented medicines. Based on the success of pooled procurement, the study authors recommended extending this approach to other programmes with high treatment costs under the NHA, which administers the Ayushman Bharat Pradhan Mantri Jan Arogya Yojana, the government's health insurance initiative.

TAKEAWAYS

- NCDs cause 66 per cent of deaths in India, highlighting a major health challenge
- Improving diagnostics at primary healthcare levels is vital for early disease detection
- The elderly population is set to more than double by 2050, calling for specialised care
- Lowering out-of-pocket expenditures and expanding insurance coverage are crucial for accessibility

EXPANDING INSURANCE COVERAGE

Private players can also help improve affordability, particularly through better insurance plans. The government's Ayushman Bharat scheme does not cover some critical areas such as OPD, dental and HIV treatment costs, and its Rs 5 lakh coverage is considered inadequate by many doctors for covering surgical as well as long-term NCD treatment costs. "Free healthcare will not work in a country with so many people, we need affordable insurance plans instead," says Dr Devi Shetty, whose Narayana Health has introduced a new health insurance policy which is being seen as the private sector's answer to Ayushman Bharat. The scheme offers a sum assured of Rs 1 crore for surgeries and Rs 5 lakh for medical management at its network hospitals for a premium of Rs 10,000 per year. While India remains on the right track—investing in greater public health infrastructure—the efforts need to be accelerated. Increasing diagnostic capacities at the primary healthcare level, improving the health worker-to-patient ratio, bringing down OOPE through increased public spending, insurance coverage and price control mechanisms are needed if we are to achieve the goal of universal health coverage by 2050. ■